

EMERGENCY FIRST AID, CPR, AED AND DIABETES MANAGEMENT TRAINED AND CERTIFIED PERSONNEL

The purpose of this Regulation is to clarify LCPS' training requirements and procedural response for medical emergencies and the care of students with diabetes.

A. Emergency First Aid, CPR, AND AED. According to [Virginia Code § 22.1-298.1\(D\)\(3\)](#), every person seeking initial licensure or renewal of a license shall provide evidence of completion of certification or training in emergency first aid, cardiopulmonary resuscitation, and the use of automated external defibrillators. The certification or training program shall: (i) be based on the current national evidence-based emergency cardiovascular care guidelines for cardiopulmonary resuscitation and the use of an automated external defibrillator, such as a program developed by the American Heart Association or the American Red Cross; and (ii) include hands-on practice of the skills necessary to perform cardiopulmonary resuscitation. The Virginia Board of Education shall provide a waiver for this requirement for any person with a disability whose disability prohibits such person from completing the certification or training.

In addition, all resource nurses, school nurses, health clinic specialists (HCS), and school nurse assistants (SNA) are certified every two years in CPR and the use of an AED. Licensed nurses receive the Basic Life Support certification, and unlicensed HCSs and SNAs receive the HeartSaver certification. All staff are certified in First Aid as part of their on-board training, and all HCSs and SNAs are recertified every two years. Training is provided by resource nurses who have been certified as instructors for the American Heart Association.

1. AED and Medical Emergency Response Teams (MERT). An AED Team consists of the school nurse or HCS, 3-4 staff trained to administer CPR and in the use of an AED, 1-2 scribes, 1-3 staff members responsible for fetching the AED, a main office secretary, and an administrator. Each school shall develop an AED Team at the start of each school year. AED team members are designated by the principal. The AED team shall hold a practice drill at least once per year using the AED trainer device and a CPR manikin.

A MERT consists of 4 staff who have successfully completed REVIVE! Training. Per the Standard Operating Procedure, all secondary schools are required to develop two MERTs at the start of each school year. The MERTs will hold a practice drill at least once per year.

When the AED Team or the MERT has been activated, the Emergency Response Form will be completed by the scribe. A copy of the form will be forwarded to the Supervisor of Student Health Services after the emergency has been resolved.

EMERGENCY FIRST AID, CPR, AED AND DIABETES MANAGEMENT TRAINED AND CERTIFIED PERSONNEL

B. Management of Diabetes in the School Setting. [Virginia Code § 54.1-2901\(A\)\(13\),\(20\),\(26\)](#) allows employees of a school board, authorized by a prescriber, with written parental permission and trained in the administration of insulin and glucagon, to administer insulin or glucagon to students who are diagnosed with diabetes.

C. Diabetes Medical Management Plan. A Diabetes Medical Management Plan (DMMP) must be completed by a health care provider and signed by the parent(s)/guardian(s) each year. The DMMP is due before the beginning of the school year or upon diagnosis. A new DMMP, an addendum, or written orders are required for additions or changes to the DMMP. It is the responsibility of the parent(s)/guardian(s) to obtain any additions or changes from the student's health care provider. The DMMP will be reviewed by the resource nurse or school nurse for clarity and completeness. The resource nurse or school nurse will contact the medical provider if there are questions about the DMMP or a need for further information related to the DMMP.

Once approved, the resource nurse or school nurse, parent(s)/guardian(s), and any unlicensed trained staff and teachers who have responsibility for the child at school shall meet to discuss the DMMP to determine individual needs of the student. This information will be incorporated and developed into a classroom Emergency Action Plan (EAP). The EAP will be reviewed with those teachers and staff who have direct care of the student. If the student rides a school bus, transportation and the bus driver will be informed of the student's diagnosis and a resource nurse or school nurse will review the EAP with the bus driver.

D. Training. LCPS, in accordance with the recommendations made by the Virginia Department of Education Diabetes Management in Schools: Manual for Unlicensed Personnel, will provide the following levels of training to ensure effective diabetes management in the school setting.

1. Level 1. Each school year all school personnel should receive training that provides a basic understanding of diabetes, how to recognize and respond to the signs and symptoms of low blood glucose (hypoglycemia) and high blood glucose (hyperglycemia), and whom to contact immediately in case of an emergency.

2. Level 2. Each school year classroom teachers and all school personnel who have responsibility for students while at school--sponsored activities shall receive Level 1 training, plus additional training to carry out their individual roles and responsibilities and to know what to do in case of a diabetes emergency. This training, however, does not include the administration of glucagon and insulin.

3. Level 3. Three or more school staff members shall receive in-depth training about diabetes and routine and emergency care for each student with diabetes

EMERGENCY FIRST AID, CPR, AED AND DIABETES MANAGEMENT TRAINED AND CERTIFIED PERSONNEL

from a registered nurse. This training enables staff to help all students with diabetes in case of emergency, and in the performance of diabetes care tasks (e.g., administering insulin, checking blood glucose levels).

Parent(s)/guardian(s) are welcome to demonstrate use of their child's continuous glucose monitor (CGM) and/or insulin pump.

Only LCPS employees or agency staff who have completed the initial four-hour insulin and glucagon training module (Level 3 Training), and the two-hour hands-on training course, in accordance with the Virginia Department of Education Manual for Training of Public School Employees in the Administration of Insulin and Glucagon, may administer insulin and glucagon to students. This training shall be provided by a registered nurse. Staff that have successfully completed the Level 3 Training may recertify each school year, by completing the Diabetes Refresher Module and the Diabetes Skills Refresher Training. If a school year passes without the staff member completing the Refresher Module and Diabetes Skills Refresher Training, staff will need to complete the four-hour Level 3 training and two-hour hands-on training course to perform diabetes care tasks and provide routine care and emergency care for students with diabetes.

Where a registered nurse is available, no employee who is not a registered nurse, shall assist with the administration of insulin or glucagon.

E. Students with Insulin Pumps and/or Continuous Glucose Monitors.

1. Use of an Insulin Pump. An insulin pump is a computerized device that is programmed to deliver small, steady doses of insulin throughout the day. Additional doses are required to cover food intake and to lower blood glucose levels.

Due to various manufacturers, the different models and functions of insulin pumps, and the continuous introduction of new models and capabilities of the pumps, parent(s)/guardian(s) are asked to perform all basic set-up and maintenance care tasks at home.

In the event a pump should malfunction, become dislodged, or come off the body, insulin will be administered in accordance with the DMMP. The parent(s)/guardian(s) will be notified. If the student has been identified, per the DMMP, as independent in insulin pump management skills to include re-insertion, the student can re-insert the infusion set/pump. If the student is unable to perform this skill and the parent(s)/guardian(s) is not available to come to school to re-insert the infusion set/pump, insulin will be administered by insulin vial and syringe, or insulin pen as per the DMMP.

EMERGENCY FIRST AID, CPR, AED AND DIABETES MANAGEMENT TRAINED AND
CERTIFIED PERSONNEL

[Virginia Code § 22.1-274.01:1\(B\)](#), states each local school board employee who is a registered nurse, licensed practical nurse, or certified nurse aide and who has been trained in the administration of insulin, including the use and insertion of insulin pumps and the administration of glucagon may assist a student who is diagnosed with diabetes and who carries an insulin pump with the insertion or re-insertion of the pump or any of its parts. Prescriber authorization and parental consent shall be obtained for any such employee to assist with the insertion or re-insertion of the pump or any of its parts. Nothing in this section shall require any employee to assist with the insertion or re-insertion of the pump or any of its parts.

2. Use of Continuous Glucose Monitors. A continuous glucose monitor (CGM) is a device that measures the blood glucose levels and trends throughout the day. The CGM is a useful tool for identifying trends and can enhance the ability of the student's diabetes health care team to make needed adjustments to the student's diabetes plan. The CGM can measure the glucose in the fluid between the cells at regular intervals and sends the current equivalent glucose level wirelessly to a monitor. The monitor may be part of the insulin pump or a separate device, which may include a smartphone that is carried or worn by the student in a pocket, backpack, or a purse. The CGM sets off an alarm when glucose levels are out of range, or when they are increasing or decreasing at a rapid rate. Appropriate actions, in accordance with the student's DMMP, should be taken when the CGM alarms. At this time, treatment decisions and diabetes care plan adjustments should not be based solely on CGM results, except if indicated on the DMMP and for the specific model of CGM, as indicated on the DMMP.

Parents/guardians are expected to perform all CGM maintenance care tasks at home to include required regular calibrations. Staff may calibrate the CGM after contacting the parent when CGM data is not aligned with blood glucose obtained via fingerstick.

F. Use of Mobile Applications. Some CGM's transmit data remotely to multiple devices at the same time via smartphone technology using an application. A parent/guardian may request that LCPS staff use the application to periodically observe their child's CGM data during the school day by completing the Parental Agreement for Continuous Glucose Monitor Mobile Applications and the Release of Information. These forms must be completed at the start of each school year.

Once the completed Parental Agreement and the Release of Information have been received and the DMMP has been approved, the following procedure will be implemented:

1. The mobile application will be downloaded onto the iPad provided by LCPS.

EMERGENCY FIRST AID, CPR, AED AND DIABETES MANAGEMENT TRAINED AND
CERTIFIED PERSONNEL

2. An email account specified for CGM mobile application invitations will be created.
3. The parent/guardian will email an invitation to download the mobile application and follow the student's data.
4. The alarm parameters on the LCPS iPad and the student's device will be aligned with the blood glucose range ordered by the medical provider on the DMMP.
5. The student's name and ID number will be used to identify the student within the mobile application. The student's picture may be added to the application with permission from the parent/guardian.
6. Health office staff, and staff with Level 3 Training will use the mobile application to observe the student's glucose level per the provider's orders in the DMMP and may periodically observe the glucose level and trends at other times during the school day. Intervention will be provided according to the provider's order in the DMMP.
7. LCPS staff will respond to alarms and follow the provider's order in the DMMP.

LCPS is not obligated to oversee the student's blood glucose levels beyond the provisions outlined in the student's 504 Plan, Individualized Education Program (IEP), and/or DMMP. Any further monitoring is provided as a voluntary support for both the student and the student's parent(s)/guardian(s) and is subject to LCPS' discretion.

Issued: 8/23/24

Leg. Refs.: [§ 22.1 - 274.](#), [VDOE Regulation](#)

Cross Ref.: [VDOE Diabetes Management in Schools: Manual for Unlicensed Personnel](#), [American Diabetes Association](#), [Policy 8420, Student Medication](#), [Regulation 8420, Student Medication](#)